NCCP 00372 · VERSION 6

Your treatment, explained

Adenocarcinoma NSCLC · combination phase · NCCP 00372 v6 · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Nintedanib + docetaxel

21-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call your oncology team immediately for fever, shaking chills, sudden illness, breathing difficulty, chest pain, significant bleeding, severe tummy pain, uncontrolled vomiting or new severe headache, confusion, seizure or vision change. Do not wait for the next appointment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp · smpc

Choose a topic to read

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Treatment stages
- Your 21-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- Supportive medicines
- Understanding the evidence
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

Three medicines. Different jobs.

To help control adenocarcinoma lung cancer after first-line chemotherapy; treatment benefit and side effects are reviewed together.

TAXANE CHEMOTHERAPY

Docetaxel

Docetaxel



Stabilises microtubules

Docetaxel prevents normal rearrangement of the cell structures needed for division. It affects cancer cells and can also suppress blood cells, irritate nerves and cause fluid retention.

Stabilises microtubules → Cancer growth disrupted → Healthy tissues may also be affected

What to watch for

Fever or feeling suddenly unwell needs urgent advice. Tell the nurse immediately about infusion symptoms or pain.

Sources: nccp

ORAL ANGIOKINASE INHIBITOR

Nintedanib

Nintedanib



Blocks VEGF, FGF and PDGF receptor signalling

Nintedanib blocks several signals involved in the growth of blood vessels that support a tumour. Its effects can also cause diarrhoea, liver changes, bleeding and wound-healing problems.

Blocks VEGF, FGF and PDGF receptor signalling → Cancer growth disrupted → Healthy tissues may also be affected

What to watch for

Call early for diarrhoea or poor fluid intake; report bleeding, yellow eyes, dark urine, severe tummy pain or new neurological symptoms urgently.

Sources: nccp · smpc

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

Treatment → review → next steps

COMBINATION PHASE

Combination phase

1–6

Day 1 docetaxel; Days 2–21 nintedanib twice daily. Up to 6 cycles.

POSSIBLE TABLET-ONLY PHASE

Possible tablet-only phase

Review

Only after at least 4 completed combination cycles and team approval: nintedanib every day in 28-day cycles. This change is not automatic.

Your team confirms each stage and discusses any changes with you.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

My treatment and safety checks

Routine safety tests are separate from scans and tumour markers, which follow your individual plan. No universal imaging or tumour-marker interval is specified.

Check	Before treatment	Treatment / review	Between visits	Phase decision	After treatment
Treatment	Confirm combination plan	Day 1 docetaxel; Days 2–21 nintedanib	Capsules twice daily	After≥4 cycles, review possible continuation	No automatic phase switch
Blood count	Baseline	Before each cycle	Around nadir / if unwell	After last combination as indicated	Follow abnormalities
Kidneys / liver / BP	Baseline bloods and BP	Bloods each cycle	BP as indicated; extra tests if needed	Check tolerance before change	Liver monthly if oral continuation
Scans / other care	Individual disease plan	No fixed scan interval	Report symptoms early	Assess benefit	Agreed follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00372 v6 · official regimen · Vargatef current EMA product information

THE WHOLE TREATMENT JOURNEY

Nintedanib + docetaxel: treatment and checks

Adenocarcinoma NSCLC · combination phase

Day 1: docetaxel drip, with no nintedanib. Days 2–21: nintedanib capsules twice daily. Repeat every 21 days for up to 6 cycles (about 18 weeks without delays); treatment may stop earlier. A later tablet-only phase needs a new team-confirmed plan.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment / review	Between visits	Phase decision	After treatment
Treatment NCCP 00372 · SMPC	Confirm combination plan	Day 1 docetaxel; Days 2–21 nintedanib	Capsules twice daily	After≥4 cycles, review possible continuation	No automatic phase switch
Blood count NCCP 00372 · SMPC	Baseline	Before each cycle	Around nadir / if unwell	After last combination as indicated	Follow abnormalities
Kidneys / liver / BP NCCP 00372 · SMPC	Baseline bloods and BP	Bloods each cycle	BP as indicated; extra tests if needed	Check tolerance before change	Liver monthly if oral continuation
Scans / other care NCCP 00372 · SMPC	Individual disease plan	No fixed scan interval	Report symptoms early	Assess benefit	Agreed follow-up

What each check is for

1

Prepare

Before treatment

To help control adenocarcinoma lung cancer after first-line chemotherapy; treatment benefit and side effects are reviewed together.

Ask / act: Confirm the aim, phase, prescription, tests and contacts.

Sources: NCCP 00372

2

Review

Before each cycle / at agreed reviews

Check benefit and tolerability

Ask / act: Report symptoms and confirm the next plan.

Sources: NCCP 00372

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

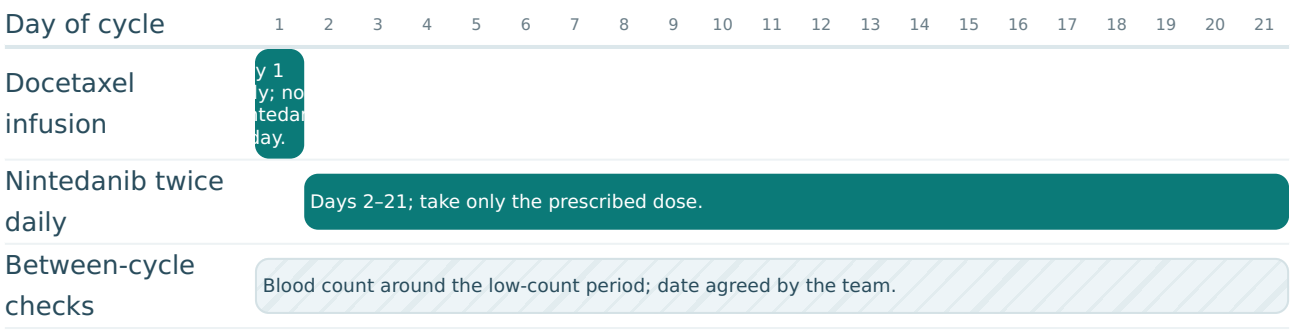
This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-02 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.30

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

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Symptoms can occur at any time: Side effects and infection do not follow a guaranteed day-by-day risk curve. Contact the team when symptoms occur.

Changes for safety: Treatment may be delayed, reduced or stopped. Do not make up missed doses or change your tablets yourself.

Why these bands stay broad: Relative days without personal dates. The team confirms actual treatment and test dates. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.

There are no guaranteed “safe days”. Call your oncology team immediately for fever, shaking chills, sudden illness, breathing difficulty, chest pain, significant bleeding, severe tummy pain, uncontrolled vomiting or new severe headache, confusion, seizure or vision change. Do not wait for the next appointment.

Read the whole cycle as text

Day 1: Docetaxel day

Docetaxel infusion after checks. Do not take nintedanib today. Follow the dexamethasone prescription. Use the current prescription and contact plan.

Day 2–21: Nintedanib at home

Take the prescribed capsule dose twice daily about 12 hours apart. Continue the agreed monitoring and support plan. Use the current prescription and contact plan.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Dexamethasone before docetaxel

Take the prescribed steroid course from the day before docetaxel. If missed, contact the unit before attending; do not invent a catch-up dose.

STEP 2

Infection prevention

The team should consider preventive G-CSF injections and arrange blood counts around the low-count period. Fever needs immediate advice.

STEP 3

Capsules

Take nintedanib about 12 hours apart with food; swallow whole. Do not open, chew or crush. If missed, take the next scheduled dose only; never double up.

STEP 4

Diarrhoea plan

Have the prescribed anti-diarrhoeal and fluid plan ready. Contact oncology early for diarrhoea, poor intake or vomiting; do not wait 7 days to seek help.

STEP 5

Medicine review

Check all medicines and supplements with oncology/pharmacy. Nintedanib has P-gp interactions; docetaxel has CYP 3 A interactions, including grapefruit concerns.

STEP 6

Pregnancy prevention

Discuss fertility and effective contraception. Nintedanib requires contraception during treatment and for at least 3 months after; follow the longest advice for your complete regimen. Vomiting/diarrhoea can reduce pill absorption.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00372 v6 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Dexamethasone before docetaxel

Take the prescribed steroid course from the day before docetaxel. If missed, contact the unit before attending; do not invent a catch-up dose.

2 Infection prevention

The team should consider preventive G-CSF injections and arrange blood counts around the low-count period. Fever needs immediate advice.

3 Capsules

Take nintedanib about 12 hours apart with food; swallow whole. Do not open, chew or crush. If missed, take the next scheduled dose only; never double up.

4 Diarrhoea plan

Have the prescribed anti-diarrhoeal and fluid plan ready. Contact oncology early for diarrhoea, poor intake or vomiting; do not wait 7 days to seek help.

5 Medicine review

Check all medicines and supplements with oncology/pharmacy. Nintedanib has P-gp interactions; docetaxel has CYP 3 A interactions, including grapefruit concerns.

6 Pregnancy prevention

Discuss fertility and effective contraception. Nintedanib requires contraception during treatment and for at least 3 months after; follow the longest advice for your complete regimen. Vomiting/diarrhoea can reduce pill absorption.

Sources: nccp · ncis · antiemetic

Understanding the evidence

The trial found a progression-free benefit overall and an overall-survival benefit in the prespecified adenocarcinoma subgroup.

The licensed/NCCP population is adenocarcinoma after first-line chemotherapy. Overall-population PFS and adenocarcinoma-subgroup OS are labelled separately; this does not establish an independent benefit from maintenance-only initiation.

Sources: trial

PREPARE FOR THE CONVERSATION

Questions worth asking

Who confirms my next dose?

Your treating team checks the prescription, tests and symptoms. A completed-cycle stamp records past treatment; it does not authorise the next dose.

What if the plan changes?

Ask the team to update your phase, medicines, test appointments and cycle record. Use the updated prescription.

When are my scans?

Your team sets a response and follow-up plan for your cancer and symptoms. This document does not invent a fixed scan or tumour-marker timetable.

> **Try explaining the plan back:** Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00372 v6. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00372 v6 · official regimen
- Vargatef current EMA product information
- NCCP antiemetic classification v 6 and oral pathway
- NCIS medical oncology antiemetic medicines
- LUME-Lung 1 primary publication

SACTCheck · Developed by Dr Paul O’Brien · Sources checked 2026-11-02.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6913/372_Nintedanib_therapy.pdf

https://www.ema.europa.eu/en/documents/product-information/vargatef-epar-product-information_en.pdf

https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_SACT_Induced_Nausea__pWgDCrm.pdf

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://pubmed.ncbi.nlm.nih.gov/24411639/